

February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
U.S. Department of Health and Human Services

RE: Request for Information; Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care [RIN 0955-AA13]

Submitted via email to www.regulations.gov

Dear Assistant Secretary Dr. Keane:

AHIP is the national association that represents health insurance plans that provide coverage, services, and solutions for over 205 million Americans through Medicare and Medicaid, employer-sponsored insurance, and the individual insurance market. AHIP appreciates the opportunity to provide comments on the Department of Health and Human Services' (HHS) Request for Information (RFI) on "Accelerating the Adoption and Use of Artificial Intelligence (AI) as Part of Clinical Care."

As Americans increasingly interact with AI in many facets of life, including across the health care system, it is important to create balanced policies that help realize the potential of AI and promote innovation, while also prioritizing safety and building trust among patients, providers, and other stakeholders. AHIP supports the Department's efforts to build on its December 2025 AI strategy by seeking public feedback on steps it can take to establish a "forward-leaning, industry-supportive, and secure approach" to accelerate the adoption and use of AI as part of clinical care. HHS can play an important role in advancing the use of AI, including improving clinical outcomes, by prioritizing coordination and consistency within the Department and across Departments, investing in AI standards development and education, and continuing to collaborate with stakeholders by participating in, and utilizing recommendations from, public-private partnerships, among other actions.

Below, we provide a snapshot of the current state of health plan use of AI to support clinical care and provide input on specific questions and topics addressed in the RFI.

Health Plan Use of AI

AI is used **to help consumers, employees, providers, and other partners in improving health outcomes and the health care experience, as well as in gaining internal operational efficiencies and reducing costs.** Examples of where AI is being used to bring value to the health care continuum include:

Clinical:

- Supporting provider partners by identifying gaps in care and potential safety issues.
- Improving predictive analytics of patient risks to enable proactive disease and care management in partnership with providers.
- Researching disease pathways, treatment options, and likely outcomes.
- Supporting customized patient care plans, early interventions, and customized treatment.
- Cleaning, integrating, normalizing, and analyzing vast amounts of data faster and more efficiently than traditional statistical methods.

Consumers:

- Offering chatbots for simple and routine member inquiries, such as customer support and administrative questions.
- Facilitating accurate and timely answers to consumers' asking more detailed or complex questions about their health benefits by equipping customer service representatives with informational support tools.
- Providing health plan apps that include pricing estimates, among other features.
- Supporting consumer appeals processing by extracting information from faxed appeals and instantly generating a fully formed digital case file, for human review.

Administrative:

- Near real-time *approvals* of prior authorizations based on appropriate documentation and member clinical history. Potential prior authorization denials based on clinical factors are always subject to review by a licensed and qualified clinician.
- Contributing to efficient claims processing and reductions in administrative costs, including flagging claims processing issues to reduce errors and support claims operators.
- Promoting payment integrity by helping to identify patterns of payment abnormalities, enabling health plans to engage in appropriate review, investigation, and corrective action.

Barriers to Private Sector Innovation

A Federal Approach

The Executive Order, *Ensuring a National Policy Framework for Artificial Intelligence*, appropriately seeks to ensure “there is a minimally burdensome national standard — not 50 discordant State ones.” A unified federal approach for AI is essential to ensure consistency, reduce duplicative compliance efforts, and promote innovation across health care. Fragmented, state-level approaches risk creating conflicting or redundant obligations that increase administrative burden and delay the deployment of responsible AI solutions. Divergence would also occur among states and federal programs such as Medicare Advantage and Part D, which are not subject to state laws governing AI use and oversight under relevant preemption provisions. Office of Science and Technology Policy (OSTP) and the Centers for Medicare & Medicaid Services (CMS) should work together to ensure that federal and state requirements are

harmonized across these programs, resolving ambiguity that could otherwise create uneven obligations or discourage innovation.

Effective coordination also depends on clear alignment within HHS agencies such as CMS, the Food & Drug Administration (FDA), and the Assistant Secretary for Technology Policy (ASTP), as well as across agencies such as HHS, the Department of Homeland Security, and the Federal Trade Commission (FTC). Overlapping requirements could create uncertainty and slow innovation. HHS can ensure a cohesive national framework in which agencies share best practices, avoid redundancy, and promote common expectations for AI governance. Continued reliance on established frameworks, such as the NIST AI Risk Management Framework, can further support consistency and predictability in implementation.

Risk-based Oversight of AI in Health Care

Federal AI oversight should be proportional to risk, allowing flexibility for low-risk uses while focusing on patient safety for high-impact applications. This approach will allow agencies to focus regulatory attention where it is needed the most, like use cases that directly impact clinical decision making, while allowing flexibility for administrative tools that pose minimal risk. A risk-based approach includes identifying, analyzing, and prioritizing risks, then developing and implementing mitigation strategies while monitoring and communicating throughout the process.

As HHS considers policies to advance clinical AI adoption, it may be helpful to recognize that these technologies operate across distinct functional contexts with differing implications for patient care and decision-making. These uses present varying levels of influence on clinical decisions and patient actions, ranging from information support to shaping potential care regimen. Recognizing these distinctions can help ensure governance and transparency requirements are calibrated appropriately to the intended use and impact of the technology.

Privacy & Security Concerns

HIPAA-compliant AI systems, including large language models (LLMs), are critical for building trust and driving consumer adoption. Consumers are more comfortable using AI tools when they know their personal health information will not be used to train models or become publicly accessible. In contrast, third-party, non-HIPAA compliant AI systems can undermine consumer confidence because they do not offer the same robust protections for Protected Health Information. More AI solutions could be leveraged to improve health care if more AI solutions offered HIPAA-compliant options. Many AI tools available for deployment today are not HIPAA-compliant, limiting the ability of HIPAA-covered entities to maximize the potential of those tools for health care and health care financing use cases.

Safety Standards & Training

The notion of “garbage in, garbage out” is critical in the AI space. Crafting appropriate prompts and assuring that patient data is communicated is critical to using AI in any clinical use case. Therefore, clinicians who will be utilizing AI as part of their practice should have appropriate training. Medical schools generally have not prepared clinicians for this new

technology and, while it is easy to develop any prompt for AI, it takes more advanced training and skill to develop effective prompts that will enable safe AI clinical support.

AHIP encourages HHS to engage with public-private partnerships to advance efforts that focus on the safety of AI-supported clinical decisions to prevent harm or death. HHS should further ensure that AI vendors transparently disclose critical information that guided the development of their health care AI products, particularly those used in clinical care that are not subject to FDA approval. Experts should identify critical elements, such as what populations were used to train the AI. Vendors should be required to publish ongoing statistics identified as critical by AI and health care experts, such as what percentage of AI diagnoses are over-ruled by human clinicians and how often AI fails to prompt clinicians for missing data critical to patient evaluation. Any AI system that influences a medical decision must be subject to the same high standards of transparency and safety, regardless of what that system is called (e.g., non-medical device, Software as a Medical Device, digital therapeutic, etc.).

Transparency & Traceability

Transparency and explainability are critical to fostering trust and accountability in the use of AI. Stakeholders must understand how AI systems are designed, validated, and governed to ensure they operate safely, and in support of high-quality care. Vendors should share what guided the development of their health care AI products, both those used in clinical care that are subject to FDA approval and those that are not. Experts should identify critical elements, such as what populations were used to train the AI. Transparency should focus on documenting model purpose, data inputs, and governance, rather than requiring public disclosure of proprietary algorithms that could compromise intellectual property or cybersecurity.

Voluntary, collaborative initiatives such as pilot programs, peer review, and regulatory sandboxes can strengthen oversight while promoting responsible innovation. These efforts enable payers, providers, and regulators to evaluate AI tools, address potential bias, and refine approaches before broad implementation. HHS should work with relevant federal agencies, including NIST, to advance voluntary transparency and explainability frameworks, such as pilot programs and sandboxes, and share model evaluation mechanisms that encourage trust and accountability across stakeholders.

AI tools used in high-risk clinical use cases warrant thoughtful governance and appropriate oversight. However, given the continuing evolution of these technologies and nuances of use cases in health care, governance approaches should be risk-based, grounded in existing applicable frameworks, and use-case appropriate. HHS should prioritize ongoing collaboration with stakeholders to advance consensus-driven standards development to promote safety, accountability, and transparency in high-risk clinical AI use cases, while remaining operationally feasible and supportive of innovation.

Adverse Events Reporting

While general transparency and explainability may be voluntary, more robust and stringent safety reporting should be required for high-risk clinical AI uses. AHIP encourages HHS to

engage with public-private partnerships to define high-risk use cases and advance efforts that monitor the safety of high-risk AI-supported clinical use cases to mitigate the risk of harm or death. This should go beyond just adverse events. For example, HHS should ensure that AI vendors transparently disclose critical information such as if AI fails to prompt clinicians for missing data critical to patient evaluation and the tool's output. Any AI system that provides healthcare information used by patients or clinicians to make high-stakes decisions must be subject to the same high standards of transparency and safety, regardless of what that system is called (e.g. non-medical device, Software as a Medical Device, digital therapeutic, etc.). Similarly, providers should also have to report adverse events or near misses in alignment with existing practices for other sentinel events in health care. HHS should then evaluate the data and publish results to inform patients and clinicians.

Regulatory, Payment Policy, or Programmatic Changes

Clinical Algorithms

The Nondiscrimination in Health Programs and Activities final rule issued in May 2024 finalized provisions related to clinical algorithms, later revised to "patient care decision support tools." This section duplicates other consumer protection provisions that prohibit discrimination in health programs and activities, creating confusion and unnecessary regulatory compliance burden. HHS should rely on existing law and partner with industry stakeholders through ongoing efforts to develop policy solutions that promote innovation, consistent industry standards, accurate data collection, and common definitions. **HHS should rescind 45 CFR § 92.210, nondiscrimination in the use of patient care decision support tools.**

Machine Translation

The Nondiscrimination final rule also sets limitations on the use of machine translation of languages by covered entities. These restrictions risk slowing the adoption of AI-powered translation tools, even as technology rapidly improves. In contrast, Executive Order No. 14224 and the Department of Justice's updated limited English proficiency (LEP) guidance encourages agencies to consider the responsible use of artificial intelligence and machine translation to bridge language barriers and improve efficiency. The DOJ guidance states: "Technological advances in translation services will permit agencies to produce cost-effective methods for bridging language barriers and reducing inefficiencies with the translation process. The Department encourages other agencies to follow its approach of considering responsible use of artificial intelligence and machine translation to communicate with individuals who are limited English proficient." Adopting a less restrictive, risk-based approach to machine translation would provide the flexibility needed to allow covered entities to utilize evolving AI technologies to better serve diverse populations.

In addition, we encourage the federal government to support HHS in its continued work with stakeholders to develop and evaluate alternative approaches to current notice requirements. Such approaches can use technology to more effectively let people know language assistance is available and how to access it, such as a universal language assistance symbol, and promote electronic access without burdensome or costly regulatory requirements. Such a symbol could

appear not only in physical locations, but also on websites and in electronic documents. AI can be used as back-end support once assistance is selected to surface the appropriate language in writing or generate speech. The regulations should not hamper advancement as technology evolves.

HHS should rescind 45 CFR § 92.201(c)(3) and align guidance related to machine translation for LEP with Executive Order 14222 and the Department of Justice guidance.

Additionally, HHS should maintain regulatory flexibility to develop meaningful access for LEP consumers that focuses on quick and streamlined access to interpretation and written translations that are supported by technology.

Software as a Service (SaaS) Payment

CMS has appropriately devoted attention to advances in technology and its growing presence in health care. At the outset, AHIP emphasizes the importance of rigorous evaluation by CMS around efficacy, safety, and quality of clinical SaaS products in its coverage determinations, a necessary and threshold factor for CMS payment. In establishing a payment framework for SaaS, we recommend that CMS adopt a balanced, evidence-driven approach based on a clear and transparent framework that seeks to anticipate future innovations. We suggest CMS first classify SaaS tools by type of solution (e.g., diagnostic, therapeutic, augmentative, patient-facing, operational) and then create a series of algorithms to associate the technology with the appropriate payment mechanism.

In general, we recommend CMS establish a unified, site-neutral framework for paying for SaaS across the PFS and OPFS. CMS payment policy should prioritize packaging when tools are integral to a service, allow time-limited add-ons or coverage with evidence while evidence is emerging, reserve separate payment for primary services, and avoid “buy-and-bill” where feasible. Valuation by CMS should consider clinician work, practice expense, malpractice, and the technology’s acquisition and capital-like costs, with periodic rebasing as efficiencies materialize. We also suggest that CMS address SaaS through value-based payment models and risk-based contracts that reward measurable outcomes and efficient care delivery, and pair payment with quality measures and interoperability/cybersecurity expectations to protect patients and ensure sustained value. **AHIP’s full response to the CMS CY2026 Outpatient Prospective Payment System (OPFS) Proposed Rule SaaS Request for Information is attached.**

HHS Support for Private Sector Activities

Standards and Frameworks

HHS can support the private sector by deferring to industry standards where they exist and funding efforts to address ongoing gaps as technology evolves.

Stakeholders in the private sector have been collaborating for several years to develop governance, ethical, and practice standards for organizations developing and deploying AI to protect consumers while fostering AI. AHIP has joined business and technology leaders as well as consumer advocates to advance principles, best practices, and standards. For example, AHIP has worked with the

Consumer Technology Association (CTA) on developing standards of trustworthiness and recommendations for bias management and continues to work with CTA to identify areas in which new standards are needed.¹²

AHIP strongly encourages use of these and other existing national frameworks and standards in developing any new federal policy. This approach would help avoid duplication and promote a streamlined oversight structure to support continued innovation. For example, the NIST AI Risk Management Framework, developed with robust input from stakeholders, including AHIP, provides a foundation for understanding and applying methods for “tiering” of risks associated with AI.³ Further, as the Administration seeks to develop and promote the innovation potential that AI offers, it is important that federal law preempt state laws. Many states are releasing new provisions and requirements that could stifle the development of AI and hamper efforts to standardize technologies. Alignment on common principles and uniform guardrails across Congress and the Administration is critical for AI to be used to its full potential.

Parallel Processing of Approval and Coverage

Through the Food and Drug Administration (FDA), private health plans have been working more closely with manufacturers to ensure sufficient data are available to support coverage decisions.

- The Center for Devices and Radiological Health (CDRH’s) **Payor Communication Task Force** was created to facilitate structured, ongoing dialogue between device manufacturers and payers— inclusive of both Medicare and private health plans. This Task Force supported manufacturers in obtaining timely feedback that may shorten the interval between FDA marketing authorization and coverage decisions.
- The **Early Payor Feedback Program**—part of the Task Force activities—enabled device manufacturers to consult with public and private payers well before final FDA clearance or approval. The intent of the program was to align evidence generation with both regulatory and coverage needs so that manufacturers could better anticipate insurer expectations regarding clinical utility, patient-centered outcomes, and real-world applicability.

This sort of parallel processing could substantially speed up the pathway for Software as a Medical Device (SaMD) to get regulatory approval, coverage determinations, and then CMS payment rates. Given recent reorganization and reductions in force, AHIP is unclear whether these programs continue to operate. Moreover, more robust collaboration that would permit data sharing during the approval process—not necessarily through the FDA, but in parallel—could even further benefit patients by bringing products to market more quickly. **We urge CMS to continue working with the FDA and even expand to the private sector parallel review**

¹ <https://shop.cta.tech/collections/standards/products/the-use-of-artificial-intelligence-in-healthcare-trustworthiness-cta-2090>

² <https://shop.cta.tech/a/downloads/-/b5481e81fe7f99aa/9d1895627bdd6e27>

³ <https://www.nist.gov/itl/ai-risk-management-framework>

programs and manufacturer guidance opportunities to ensure all payers, public and private, have the information necessary to make informed coverage decisions.

Promoting Adoption & Use

Smaller hospitals, providers, health plans and the consumers and patients they serve could benefit greatly from AI tools. However, cost-prohibitive pricing from AI companies and lack of resources could limit or even prohibit adoption by the very organizations that could benefit the most. Additionally, to support smaller health care entities in their AI journeys caution should be taken that any guardrails intended to ensure adequate data sets (*e.g.*, size, diversity, etc.) do not penalize organizations that may have appropriate purposes for training AI systems on relatively small inhouse data sets. HHS can support these organizations by establishing AI resource centers and/or promoting credible technology assessment organizations, such as the [Petersen Health Technology Institute](#), to demonstrate effectiveness of AI tools accessible to small and medium size health care organizations and offering access to datasets and support for AI research.

Advancing Public-Private Efforts

AHIP believes public-private partnerships and collaboration can play an important role in facilitating data-driven efforts to advance AI. **HHS should promote ongoing opportunities for public-private evidence generation and dissemination on best practices to support successful AI development and use.** This approach could include partnering with stakeholders to better understand potential applications, use cases, and risks associated with AI. These findings should be used to guide the development of best practices, guidelines, and standards and prioritize resource allocation to ensure investments are high-value for the health care sector.

Conclusion

Thank you for your consideration of our input. AHIP appreciates your efforts to engage stakeholders as you identify priorities for future Department action. We look forward to working with you to advance AI, including opportunities to support innovation in clinical care. If you have questions, please contact me at dlloyd@ahip.org or 202-778-3246.

Sincerely,



Danielle A. Lloyd
Senior Vice President, Private Market Innovations & Quality Initiatives

REQUEST FOR INFORMATION ON SOFTWARE AS A SERVICE (SAAS)

AHIP recommends that CMS adopt a balanced, evidence-driven, and incremental approach to establishing payment for Software as a Service (SaaS). We recognize the growth of evidence-based digital tools that can improve patient outcomes, reduce administrative burden, and advance efficiency in care delivery. We also agree that cost should not be a barrier to providers and patients accessing proven, high-value technologies. However, CMS should proceed thoughtfully to create a framework that will protect patients, ensure consistent access, guard against fraud, waste and abuse, and stand the test of time given the range of innovations being introduced. The framework should also seek to improve health care affordability for beneficiaries and taxpayers and avoid approaches that dramatically increase manufacturers' profits at the expense of seniors and taxpayers. We believe this will take more than marginal change including rethinking existing CMS coverage and payment constructs.

In considering how to pay for SaaS, CMS should establish policies that promote access to new technologies that are medically reasonable and necessary, meaningfully improve diagnosis or treatment, and decrease or do not significantly increase costs for beneficiaries, taxpayers, or the program. CMS should also avoid policies that create disincentives for advancing new technologies and improving existing products. Moreover, CMS should consider the broader value of technology, including quality, outcomes, efficiency, and patient experience. Digital health technology, including SaaS and artificial intelligence (AI), is not simply a supply like a syringe or the paper a medical record is printed on; these tools are reshaping the world we live in. Medicare policies must contemplate the role of digital health technology not just as inputs into its valuation scheme, but rather as infrastructure that influences clinical decision-making, clinical care delivery, patient experience and engagement, and performance across systems.

The novelty of these products, rapid proliferation of new developments, and resulting changes to health care present both new opportunities and new challenges. Unlike innovation in other spaces such as pharmaceuticals, software consistently evolves through iterative updates, including those pushed directly by manufacturers into systems, and advances in machine learning. These tools present vast opportunities to improve patient outcomes, reduce preventable medical errors, reduce administrative frustrations for providers and patients, increase efficiency, and reduce costs. At the same time, they present the risk of reducing the quality of care and increasing costs to drive manufacturer profits for unproven interventions. A strong quality and monitoring framework is important to ensure tools continue to meet the standards for coverage and payment. In addition to monitoring quality and safety, changes to CMS payment policies may be warranted even after a product receives coverage and valuation. For example, to the extent a product drives efficiency gains over time, CMS payment policy may need to be revisited to adjust values downward.

Below, we have chosen not to adhere specifically to the order of the questions presented by CMS but do generally answer the preponderance of them. We thought it was important to first discuss

overarching definitional issues and several precursor considerations including coverage, privacy and security, and quality before moving to a set of frameworks for identifying the type of tool, and potential CMS payment mechanisms, valuation methods, and quality mechanisms.

Definitions

At the outset, AHIP notes that, while the term “software as a service” is widely used, there is no clear definition within the context of Medicare payment policy. Clear and consistent definitions go beyond semantics and instead have critical implications for purposes of coverage, coding, and payment policies. SaaS at its core is simply the delivery model by which the tool is accessed by the end-user, most commonly via the internet or an app, but can be incorporated directly into hardware. In health care, this could encompass anything ranging from a patient engagement tool, such as a portal, to a prescription digital therapeutic. The range of tools captured under the SaaS model in health care is extensive and therefore requires consideration to account for differences in deployment and use. For example, specific considerations may apply when SaaS incorporates machine learning or other forms of artificial intelligence. In our comments, we generally define SaaS broadly; however, at times we specifically note how Software as a Medical Device (SaMD), as a subset of SaaS, many need special consideration. **We urge CMS to establish clear definitions as it moves forward with coverage and payment policies in this area.**

Coverage

Before determining payment, CMS must establish coverage for certain items and services used to diagnose, treat, or prevent a medical condition. There are unique aspects of SaaS that should be considered as part of this process that then impact later payment decisions.

Evidence

Products of a clinical nature (e.g., apps for diabetes management or tools that detect cardiac conditions) must be rigorously evaluated for coverage before reimbursement is established by CMS. CMS must consider whether there is evidence of a product’s safety, effectiveness, and appropriateness to establish coverage of a new technology is reasonable and necessary. Medicare does not cover items that are experimental, investigational, or not supported by adequate evidence. Private health plans also conduct these types of reviews in determining whether to establish coverage but often struggle with obtaining sufficient evidence from manufacturers.

While randomized control trials (RCTs) remain the gold standard, we recognize that RCTs are expensive, time-consuming, and difficult for digital tools that integrate artificial intelligence and evolve rapidly. In cases where RCT data are not available, CMS should require alternate evidence such as retrospective data set validation, prospective observational studies, real-world validation studies, and human factors testing. Sufficient evidence must demonstrate to CMS safety, clinically meaningful and sustained benefits, cost effectiveness (recognizing CMS has statutory limitations), and applicability and consistent access across populations. Well-designed, prospective cohort studies with appropriate size and follow-up from real world evidence data can mitigate limitations in the evidence base.

Through the Food and Drug Administration (FDA), private health plans have been working more closely with manufacturers to ensure sufficient data are available to support coverage decisions.

- The Center for Devices and Radiological Health (CDRH's) **Payor Communication Task Force** was created to facilitate structured, ongoing dialogue between device manufacturers and payers— inclusive of both Medicare and private health plans. This Task Force supported manufacturers in obtaining timely feedback that may shorten the interval between FDA marketing authorization and coverage decisions.
- The **Early Payor Feedback Program**—part of the Task Force activities—enabled device manufacturers to consult with public and private payers well before final FDA clearance or approval. The intent of the program was to align evidence generation with both regulatory and coverage needs so that manufacturers could better anticipate insurer expectations regarding clinical utility, patient-centered outcomes, and real-world applicability.

This sort of parallel processing could substantially speed up the pathway for Software as a Medical Device (SaMD) to get regulatory approval, coverage determinations, and then payment rates. Given recent reorganization and reductions in force, AHIP is unclear whether these programs continue to operate. **We urge CMS to continue working with the FDA on parallel review programs and manufacturer guidance opportunities to ensure all payers, public and private, have the information necessary to make informed coverage decisions.**

We also encourage CMS to develop a framework for the ongoing monitoring of SaMD from a coverage standpoint. Unlike traditional SaaS platforms, AI-enabled tools can continuously learn, update outputs, and perform predictive or diagnostic functions that can directly influence clinical decisions. These capabilities may require not only additional evidence to ensure appropriate integration into CMS coverage and payment policy, but also more frequent monitoring. The FDA has created a Predetermined Change Control Plans that recognize that an item or service that integrates machine learning may change over time. CMS will need to consider policies that permit a certain level of change without re-review (e.g., an annual update of data sources and normalization) and a process to routinely re-review items and services that evolve beyond this level of permitted change. **CMS should seek guidance from stakeholders on a framework and process for the review of items and services that incorporate machine learning.**

Clinical Guidelines

Physician specialty societies also play a role in vetting technology and disseminating information on its use by integrating it into clinical guidelines, or other related guidance, establishing its use as part of the standard of care. Specialty society guidance can be particularly salient where the software is not classified as SaMD and thus is not subject to vetting under the FDA approval and regulatory pipeline. However, there is minimal evidence that SaaS-based digital health tools—such as digital therapeutics, remote monitoring platforms, or AI-driven decision aids—are explicitly referenced or incorporated into the majority of such clinical guidelines. AHIP is aware of some examples in the app space such as the American Physical Therapy Association's

(APTA)⁸ Digital Health App Formulary in partnership with ORCHA and the American Psychiatric Association’s App Advisor.⁹ While the apps themselves may be free or paid directly by the payer, it is still important to understand whether physicians are expected to use such technologies as part of a service and to consider the time and intensity of, for example, reviewing the data output as part of physician compensation by CMS.

Notwithstanding SaaS classification as a SaMD, it is important for CMS, and private payers, to understand whether a technology is deployed by only a small set of early adopters or is more widely deployed in establishing coverage and payment policies. The incorporation of technology into medical specialty guidelines demonstrates that adoption of a technology is considered part of the standard of care, which is a significant factor in coverage. It also provides key information in determining how to pay for the technology. When not all physicians or outpatient departments deploy the solution embedded into clinical guidelines, despite it being the standard of care, there are policy mechanisms that can be used by CMS to encourage universal adoption. For example, in moving from computed radiography (CR) to digital radiography (DR), CMS began to reduce reimbursement for CR to incentivize the switch to DR in 2017. A similar approach could be used by CMS for other technologies that are commonplace but not universally adopted.

In contrast, if a digital solution is novel and not yet part of the standard of care, a different approach may be warranted whereby a CMS could utilize a simple add-on code that it only covers under certain circumstances and monitor over time. This could be as part of a coverage with evidence program or a temporary new technology add-on payment to ensure access prior to a technology being packaged. Payment policies are not just about the rate— what codes must be billed as part of the service, whether it is paired with a quality measure, whether prior authorization is required— are all tools available to drive care delivery transformation. **CMS should work with specialty societies to ensure SaaS-based digital health tools are integrated into their clinical guidelines or in other related guidance.**

In order for specialty societies to accomplish this, however, they need more information on the digital health tools. There are several organizations seeking to evaluate digital health tools that could be helpful to CMS, private payers, and providers. The [Digital Medicine Society](#) (DiMe) has begun work to create methodologies and frameworks for evaluating patient-facing digital health software products, which may include SaaS, including toolkits that can be used as a pre-assessment guide for developers to assess their product’s clinical quality. The [Peterson Health Technology Institute](#) (PHTI) focuses on digital/SaaS health technologies evaluating them on three dimensions: clinical effectiveness, cost-effectiveness and budget impact, and overall value for patients, providers, and payers. ECRI and Hayes, now part of [Symplr](#), are both organizations that provide evidence-based assessments of healthcare technologies and services. **CMS should consider ways in which it can support outside efforts to assess technologies across a range of factors to help determine if they are reasonable and necessary as well as provide insight into valuation and appropriate payment structures.**

⁸ <https://www.apta.org/your-practice/practice-models-and-settings/digital-health-in-practice?>

⁹ <https://www.psychiatry.org/psychiatrists/practice/mental-health-apps>

Benefit Category

We also note that CMS has benefit category constraints. CMS, for example, had to cover and pay for RelieVRx (HCPCS E1905), a virtual reality headset with a sequential set of immersive experiences used in Cognitive Behavioral Therapy (CBT), under durable medical equipment. In its coverage decision, CMS provided that “the medical software and the device on which it is housed are so integral to each other that we consider them to be one whole device, not software and a separate device.” This will not always be the case. Especially as we move toward more autonomous AI where the software is doing the bulk of the work and the human in the loop is simply part of quality control. Without a clear benefit category, CMS will continue to have to wedge innovations into existing benefit categories. With a benefit category, CMS could also then create a provider type, conditions for coverage, and cost reporting requirements, which we discuss later. **We urge CMS to seek Congressional authority for a new benefit category.**

Privacy and Security

Technology evaluation, whether for coverage or payment, should include a thorough review of privacy and data security considerations. Many new devices, for example, are wirelessly connected to share data and assess functionality, which poses both security and privacy concerns. CMS should also work with other agencies such as ASTP/ONC and the Cybersecurity and Infrastructure Security Agency (CISA) to assess other critical aspects of SaaS performance. While SaaS has great potential to improve care, it also introduces new risks to patient safety and privacy. For example, SaaS should be tested and potentially certified by ASTP to promote interoperability in a manner that is consistent with patient safety and privacy. Confirming information can be shared between outside software programs and a person’s electronic medical record in a secure manner is essential to promoting care coordination, avoiding errors, and allowing a patient to have control over their complete medical record. ASTP could also update the SAFER Guides to include SaaS safety.

Quality

Whether CMS’s payments are made within FFS or through APMs, CMS should consider how to evaluate the quality of care associated with the use of SaaS. As noted below, SaaS tools could significantly disrupt how healthcare is delivered. Quality measures can ensure that the software performs as intended in the real-world and that it supports accurate, safe, and evidence-based care rather than introducing errors or creating gaps in care. CMS should consider developing measures that assess domains such as patient safety, clinical outcomes, and patient experience. Performance measures could assess if SaaS contributes to improved outcomes on current quality measures. Through the establishment of nationwide interoperability, stratified measures could be utilized to provide an opportunity to look at services that rely on SaaS and those that do not. Additionally, measures could directly assess SaaS performance, including assessing patient experience with patient-facing tools. Information on how use of a SaaS tool improves outcomes will provide needed information promote adoption of these services when they improve care, as well as information to help monitor safety risks.

Type of Technology and Use

There are many types of SaaS-based digital health solutions. Below we propose a framework for CMS to use classifying the types of solutions, which we believe will help identify appropriate CMS payment mechanism depending on the type of solution:

Exhibit 1:

Clinical Tools: Tools akin to medical services that diagnose, treat, or prevent disease or injury.

- **Diagnostics** – Disease detection/monitoring (e.g., arrhythmia AI, radiology image analysis).
- **Treatment / Therapeutics** – Active treatment delivery or digital therapeutics (e.g., insulin dosing algorithms, prescription digital therapeutics for psychosis).

Augmentative Tools: Tools that do not perform medical services but support clinical care delivery by aiding clinicians providing a medical service.

- **Clinical Decision Support (CDS)**– Provider-facing workflow integration (e.g., AI CDS in EHRs).
- **Supplies** – Minor items that aide in the provision of care (e.g., smart thermometers).
- **Adjunctive Care** – Supportive, supervised interventions by physicians outside of the clinical setting (e.g., remote monitoring, mental health cognitive behavioral therapy. This captures the administration, not the solution, which is captured below.

Patient-Facing Tools: Tools primarily designed for use by a patient outside of a clinical setting, often to generate data, promote engagement, or support management.

- **Prevention & Engagement** – Connected devices, chatbots, and apps that collect and transmit patient-generated data (e.g., wearables, smart inhalers, smart scales, wired clothing, pain management apps) for wellness, lifestyle modification, adherence, and patient engagement tools. This captures the solution, not the administration, which is captured above.

Operations Tools: Tools that promote improvements in administrative functions or population-level care management.

- **Population Health & Analytics** – Risk stratification, predictive modeling, outcomes measurement.
Administrative & Operations – Non-clinical or back-office tools and operational AI (e.g., staffing optimization, scheduling, claims efficiencies).

These descriptions are not perfectly exclusive. For example, some decision-support tools straddle both Clinical and Administrative Support (e.g., AI systems that optimize imaging workflow and simultaneously flag critical findings). As another example, **telehealth platforms are at their**

core are communication platforms that would fall under operational tools. However, such platforms may also come with clinical and other administrative enhancements such as triage chatbots, CDS, remote monitoring, and billing and population health tools. Thus, how such platforms are addressed will depend on the totality of the services offered.

Payment

Principles

There are several high-level principles CMS should consider as it develops its SaaS payment policies including promoting sustainable and transparent frameworks, prioritizing value and affordability, taking a site neutral approach to the extent possible, packaging items and services where practicable, and seeking to avoid “buy and bill” structures.

Transparent, Sustainable Framework

CMS should establish a transparent and sustainable framework for payment of SaaS-based tools. The New Technology Add-On Payment (NTAP) under the Inpatient Prospective Payment System (IPPS) offers a useful precedent in that it sets clear eligibility criteria and offers opportunity for stakeholder input. Similarly, CMS should continue to anchor any SaaS payment framework within existing annual rulemaking cycles. Embedding SaaS policy within annual rulemaking provides transparency and ensures stakeholders have a regular opportunity to review criteria, suggest updates, and recommend refinements as technologies evolve. A durable framework with annual rulemaking as its foundation would allow CMS to: set transparent criteria for initial coverage and enhanced payment; revisit those criteria each year with public input, ensure policies adapt to clinical evidence and market experience; where applicable, provide a clear glide path for SaaS tools to transition from temporary mechanisms into permanent payment structures across settings; and, support alignment with other payers, who often consider Medicare’s rulemaking in their analyses. By embedding SaaS payment in the annual rulemaking process, CMS can foster innovation while ensuring consistency, sustainability, and transparency.

Prioritize Value and Affordability

CMS should embrace technologies that have strong evidence of patient benefit and can be delivered safely at a lower cost than legacy treatments for the same condition. To guard against inadvertently inflating costs by paying for new treatments in a manner that redirects savings to manufacturers, providers or other actors, CMS should use its authority to ensure savings gained are shared with beneficiaries and taxpayers.

Site Neutral Approach

AHIP calls attention to CMS’s bifurcated approach to payment for facility and professional services that vary rates based on site of service even when the same items and services are furnished. As CMS embarks upon establishing payment policy for these novel digital solutions, we urge CMS to contemplate a framework that can be applied consistently across both the OPFS

and PFS. While we understand why CMS has separate RFIs, given the reality that incremental change will be necessary before more sweeping changes, we believe the agency should look toward site neutral payment policies that neither incentivize beneficiaries to seek care in one setting or the other or influence where providers offer items or services.

In particular, CMS should look to eliminate its current approach that bifurcates direct and indirect non-physician costs by using practice expense (PE) relative value units (RVUs) under the PFS and ambulatory packaging classifications (APCs) under the OPPS. Instead, we encourage adoption by CMS of a unified approach to valuing SaaS products under both the PFS and OPPS, recognizing that the cost of acquiring, licensing, and maintaining the same technology should not change based on whether it is used in a hospital outpatient department or a physician office. If the underlying technology is identical in functionality and clinical use, the cost profile will be the same regardless of care setting and thus the use of separate payment policies by CMS are unwarranted. While broader harmonization of the full CMS payment systems may take more time given existing complexities embedded in payment policy, **CMS should not miss this opportunity to harmonize its methodology for capturing direct and indirect costs of SaaS where practicable.** For example, a virtual reality treatment using a headset and software should have the same payment from CMS whether furnished by a physician in an office or by a physician in an outpatient department. CMS's attention to developing a unified framework for SaaS payment presents a critical opportunity to set the framework for its payment policy on the right path from the outset, rather than perpetuating the broken and inconsistent structures embedded in its payment systems today.

Packaging

In general, we believe that it makes sense for CMS to package codes together where integral to a service. By doing so, CMS allows providers greater leverage with manufacturers to hold down costs compared to separate or pass through payments. However, we recognize that the use of certain technologies is not universal yet, and thus the use by CMS of packaging policies may overpay in scenarios where the technology is not used. As suggested above, we believe it will be important for CMS to set a framework going forward. Per-use payments by CMS can be problematic in certain circumstances. For example, per-use payments do not fully recognize the scalability of AI and its various uses in health care, such as its ability to generate outputs automatically without clinical decision making, and moreover, can create risk of overuse. As AI continues to improve and strives to automate more tasks traditionally performed by clinical staff and physicians, it can lead to more efficient practice of medicine and truncate the time clinicians spend on diagnostics, screening, prognosis, or other functions.

CMS should also consider alternatives to avoid "buy and bill" for solutions. This requires administrative work and risk for physicians and providers. It also creates an incentive for them to purchase higher cost items in order to gain greater margins when payment is structured around a percent of the total. CMS should consider direct payment for certain technologies, such as apps and in-home equipment, where it is not critical for the physician or provider to purchase the item and furnish it to the beneficiary. We recognize that the current benefit categories and provider definitions that CMS must utilize may impede this today and an enrollment system would need

to be established by CMS for solutions providers, but CMS should be looking to the future and not wedded to today's payment options.

Buy and Bill Approach

CMS should also consider alternatives to avoid “buy and bill” for solutions. This requires administrative work and risk for physicians and providers. It also creates an incentive for them to purchase higher cost items in order to gain greater margins when payment is structured around a percent of the total. CMS should consider direct payment for certain technologies, such as apps and in-home equipment, where it is not critical for the physician or provider to purchase the item and furnish it to the beneficiary. We recognize that the current benefit categories and provider definitions may impede this today and an enrollment system would need to be established for solutions providers, but we should be looking to the future and not wedded to today's payment options.

Payment Mechanisms

Below, we outline some of the potential payment mechanisms for CMS to use and suggest the payment approach appropriate for CMS to use with respect to various digital solution types.

Exhibit 2: Potential CMS Payment Mechanisms

1. Practice Expense / Bundled Input

- **Definition:** Digital solutions that are considered part of the normal cost of delivering a covered service, not separately identifiable.
- **Examples:** EHR-integrated clinical decision support used during an office visit; AI workflow tools used by radiologists to sort images; supplies like a smart thermometer or connected scale readings incorporated into routine visits.
- **CMS Payment Approach:** Folded into RVUs under the Physician Fee Schedule or bundled into DRG/APC rates for hospital services.

2. Coverage with Evidence Development (CED) / Restricted Coverage

- **Definition:** Temporary coverage for digital solutions requiring further validation of safety, effectiveness, or outcomes.
- **Examples:** Early-stage AI diagnostics; prescription digital therapeutics still generating outcomes data.
- **CMS Payment Approach:** Add-on payment or restricted HCPCS/CPT code tied to registry or mandated evidence collection.

3. Transitional Add-On Payments

- **Definition:** Short-term supplemental payments for emerging technologies not yet priced into standard payment systems.
- **Examples:** AI-enabled imaging triage tool billed under NTAP (inpatient) or Transitional Pass-Through (outpatient).
- **CMS Payment Approach:** Extra payment on top of DRG/APC until sufficient evidence/pricing data are available.

4. Permanent Add-On / New Codes

- **Definition:** Long-term recognition of a digital solution via new CPT or HCPCS codes. Add-on codes are appropriate for services that, when provided, are in conjunction with a separately payable service particularly when the service may or may not be provided with the primary service.
- **Examples:** Category I CPT codes for AI-based diagnostic tests; permanent HCPCS codes for FDA-approved digital therapeutics.
- **CMS Payment Approach:** Establish permanent fee schedule rates.

5. Packaging into Existing Services

- **Definition:** Digital solutions that are distinct from the base service but functionally supportive and therefore logically bundled into the payment for a group of services. These are solutions that are ancillary, supportive, dependent, or adjunctive to a separately payable service.
- **Examples:** AI-driven image enhancement or triage packaged into radiology interpretation payment; digital pre-op surgical planning software bundled into orthopedic surgery payment; AI-assisted anesthesia monitoring included in a surgical global fee.
- **CMS Payment Approach:** Provider reimbursed a single rate based on the presence of codes for both the primary and packaged items and services.

6. Separate Payment

- **Definition:** Applied when the digital solution itself is the primary service delivered to the patient or the focal point of the visit.
- **Examples:** FDA-cleared prescription digital therapeutics; AI-enabled diagnostic tests ordered by a physician and billed separately.
- **CMS Payment Approach:** Standalone CPT/HCPCS code with direct reimbursement. However, other supportive items and services may be packaged with this solution as the primary service.

7. Alternative Payment Models (APMs)

- **Definition:** Digital solutions are incorporated into broader risk-bearing or value-based contracts rather than fee-for-service codes.
- **Examples:** Predictive analytics guiding ACO care coordination or remote monitoring platforms through prevention and wellness technologies to manage chronic conditions.
- **CMS Payment Approach:** Payment through global budgets, bundled payments, or shared savings rather than creating new FFS codes and payments; digital solutions funded through the provider's investment to meet quality/cost benchmarks.

8. Direct Manufacturer Payment

- **Definition:** For items and services that are not used in the office/facility but rather used by the patient with the physician's role as supervisory such as monitoring data and outcomes.
- **Examples:** Nutrition apps, wired clothing, or a smart toilet

- **CMS Payment Approach:** Medicare could request authority for a new benefit category from Congress and establish a new provider type.

In general, diagnostics and treatments lend themselves to separate payment by CMS as they can be stand-alone or primary-driver services. However, CMS could package diagnostics with other services depending on the care pathway and could package other services with treatments that are primary drivers. Clinical support and supplies lend themselves to either packaging by CMS, as integral to a primary service, or CMS using add-on codes if the primary service does not adequately capture the costs. An administrative tool lends itself to the use by CMS of a relative value adjustment; meaning it is not packaged per se but rather reflected in the existing item or service code and payment. Adjunctive lends itself to direct payment of the technology by CMS and administration codes for the physician or provider. Alternative payment mechanisms could be helpful to CMS in efforts to create incentives to use certain solutions without creating specific payment codes. Below we capture a framework for the first pass at mapping a digital solution to potential CMS payment mechanisms.

Exhibit 3: Crosswalk: Digital Health Solution Types vs. CMS Payment Options

	Solution Type	PE	CED	NTAP	Add-On	Packaging	Separate Payment	APMs	Direct to Manufacturer
Clinical Tools	Diagnostics	✓	✓	✓	✓	✓	✓	✓	✗
	Treatment / Therapeutics	✗	✓	✓	✓	✗	✓	✓	✗
Augmentative Tools	Clinical Support	✓	✗	✗	✗	✓	✗	✓	✗
	Supplies	✓	✗	✗	✓	✓	✗	✓	✓
	Adjunctive Care	✗	✓	✗	✓	✗	✓	✓	✗
Patient-Facing Tools	Prevention & Engagement (lifestyle, adherence)	✓	✓	✗	✓	✗	✓	✓	✓
Operational Tools	Analytics & Population Health	✗	✗	✗	✗	✗	✗	✓	✗
	Administration & Operations	✓	✗	✗	✗	✗	✗	✓	✗

There are many facets that should be considered by CMS when narrowing down these options to a specific payment approach. The mapping by CMS of particular digital health tools to certain payment models should consider the maturity of the solution, clinical role, relationship to Medicare benefit categories, level of autonomy and other factors. To summarize some of the logic underlying our recommendation to CMS:

1. Is it the primary service? → Separate payment.
2. Is it supportive? → Minor = Practice Expense; Major = Packaging/Add-on.
3. Is it mature/ubiquitous? → Practice expense / Packaging
4. Is it emerging/uncertain? → Coverage with Evidence or Temporary Add-On.
5. Is it validated? → Permanent Add-On Code.
6. Does it only function at the population/system level? → APM.

Below we begin to build out a decision tree that considers some of these factors to arrive at a final determination of a payment mechanism for CMS to use.

Exhibit 4: Algorithm for Determining CMS Payment Mechanism for Digital Health Payment

Step 1. Is the digital solution the primary service being delivered?

If YES → Separate Payment by CMS (initially).

Example: FDA-cleared prescription digital therapeutic.

Cost impact:

- If it **adds costs**: Allow separate payment at first (often under Coverage with Evidence or Temporary Add-On) but tie to outcomes data collection.
- If it **replaces an existing service**: Start with dual payments (old + new), then **gradually phase down the old** as evidence shows equivalence or superiority. *Example:* AI-enabled pathology tool replacing manual reads — could be paid separately at first by CMS, but CMS could lower pathology professional fees over time to drive substitution.
- If it **lowers costs directly**: CMS should consider **rebasings global rates** downwards over time while preserving provider margin during the adoption phase.

Step 2. Is the digital solution supportive of another covered service?

If NO → Population/System-Level Tool → CMS use of APM.

Example: Predictive analytics guiding ACO care management.

Cost impact:

- If **adds cost but saves downstream**: CMS should build into shared savings/risk-based payments.
- If **replaces manual care management**: CMS should reduce practice expense assumptions in APM benchmarks accordingly.

If YES → Go to Step 3.

Step 3. Is the supportive solution minor and widely used (like a supply or workflow tool)?

If YES → Practice Expense / CMS should utilize Bundled Input.

Cost impact:

- If **low-cost and supportive**: Absorb into CMS's practice expense valuation.
- If **replaces manual labor**: CMS should adjust RVUs downward for the replaced inputs (e.g., radiology AI replacing junior radiologist prep work).
- If **drives net savings**: CMS should allow it to remain bundled; savings flow through rebasing by CMS of practice expense inputs over time.

If NO (distinct clinical contribution) → Go to Step 4.

Step 4. Does the solution need further evidence?

If YES → CMS should utilize Coverage with Evidence Development (CED) or Temporary Add-On (NTAP/TPT).

Cost impact:

- If **adds new cost without proven benefit**: CMS should cover only with registry requirements and sunset unless value is shown.
- If **replaces existing service**: CMS should keep both covered temporarily, then phase down the old service fee if the digital proves non-inferior or superior.

If NO (validated use) → Go to Step 5.

Step 5. Should it be packaged into the base service or paid separately?

If supportive but distinct → CMS should utilize packaging.

Cost impact:

- If **adds cost**: CMS should temporarily increase bundled rate to reflect adoption, then rebase as costs normalize.
- If **replaces existing steps**: CMS should lower the base rate inputs for the replaced elements while holding provider margins steady in transition.

If primary or standalone → Permanent Add-On / CMS should Utilize Separate Code.

Cost impact:

- If **adds cost and value**: CMS should pay separately but monitor for overutilization.
- If **replaces existing service**: CMS should shift payment by reducing the old code valuation (e.g., imaging read fee) and transferring value to the digital service code.
- If **lowers cost significantly**: CMS should rebase system-wide DRG/APC or PFS rates downward once adoption stabilizes.

Step 6. If the solution only functions at population/system level (no direct patient billing pathway).

Payment → CMS should utilize Alternative Payment Models.

Cost impact:

- If **adds cost but reduces downstream utilization**: CMS should fund through shared savings or capitation.
- If **replaces manual workflows (e.g., nurse outreach)**: CMS should adjust benchmarks downward as adoption scales.

We note that given the innovative nature of many of these technologies and the rapid rate at which they evolve, CMS may need to more heavily rely on the coverage with evidence or temporary add-on pathway at first to develop the claims and cost data needed to support a pathway to permanent coverage and payment.

Capital

The framework above assumes the additional costs to the system are on margin. However, CMS should consider the possibility of large-scale, capital-intensive technologies. Under the OPFS there is not an explicit capital payment like under the inpatient PPS; instead, acquisition costs for large-scale inputs are bundled with operating costs. However, outpatient departments have cost

reports where they are able to apportion costs across services through revenue codes to defray capital costs. Nor does the current PE valuation methodology per se reflect such capital investments. For example, if future innovation enables deployment of robotic systems that rely on SaaS and perform functions traditionally furnished by nursing staff, the costs of acquiring, licensing, and amortizing such capital investment would not be fully incorporated under the current PE framework.

In such cases, CMS's existing methodology would undervalue the investment. Temporary policies such as, or akin to, the New Technology Add-on Payment (NTAP) provide a mechanism for CMS to recognize new items; however, these approaches are narrow in scope, temporary, and may not be sustainable for integrating capital-based technologies. As technology continues to evolve in ways that blur the line between labor and capital, e.g., substituting staff time with automation, the current CMS methodology risks lagging behind innovation and failing to adequately capture resource costs.

Moreover, both the OPPS and PFS are budget neutral systems. The only way to increase the budget is through the update systems—the market basket under OPPS and the statutorily fixed conversion factor increases under PFS. Under the OPPS the market basket is a Laspeyres index that measures the change in the cost of a fixed “basket” of inputs over time. It is rare that the Bureau of Labor Statistics makes an adjustment to its indices for major net-new items (a rare example of when it did was when seat belts were introduced into cars). It did not account for, for example, the introduction of electronic health records. In part, this created the need for Congressional intervention through the Meaningful Use program. **CMS should keep in mind the constraints of the payment update systems and consider budget neutrality adjustments (both up and down) as new innovations are introduced in healthcare.**

Given the budget neutral system and the lack of a mechanism to increase payments via the update factors, we are concerned that, given the role of relative values in CMS's payment methodology, relative value shifts in favor of technology-heavy services will result in an imbalance across specialties. We are particularly concerned there may be a detrimental impact on primary care versus proceduralists. It is possible that more of the solutions primary care will use will be expected by CMS's methodology to be absorbed into the notion of practice expense or packaged, while proceduralist may more often rely on solutions that are likely to have add-on codes or are paid separately by CMS. **CMS should conduct ongoing analyses as we move forward to ensure it does not unintentionally erode overall payments to primary care and does not discourage the use of digital health solutions by primary care physicians.**

Valuation

Once the appropriate payment mechanism is selected by CMS, the next step is to determine its value. We believe the general construct of physician payment valuation is applicable to CMS use—Practice Expense (PE), Work, and Malpractice values. However, we believe an additional component is needed for CMS use that considers the cost of the technology itself from a capital perspective. While some digital solutions may have de minimis costs, some may be significant as innovations evolve.

It is important to note that there is an interplay between these four factors. The factors may move independently. One advantage of this framework for valuation by CMS is that it does not just rely on manufacturers' price but balances the potential impact of the solution on the factors—capturing both cost increase and efficiencies gained.

Ideally, increased efficiency and improved outcomes should reduce costs and produce savings in the long run. CMS must monitor the impact of establishing reimbursement for new technologies to ensure that AI is driving down costs and improving outcomes, rather than increasing overall costs or leading to overutilization. Moreover, while reimbursement by CMS may be warranted to offset the costs of acquisition of AI to avoid disincentivizing use of evidence-based technology, as costs decline due to new competition or changes in the market, CMS's reimbursement must be re-evaluated and adjusted commensurate with developments.

Digital Solution Acquisition

Currently, given the novel innovations, there is not much data on which to assess value. If CMS were able to create a new benefit category, provider type, and conditions for coverage, it could also create a cost reporting system to help establish payment rates. Otherwise, CMS could rely on provider costs captured in the hospital cost reports. This option has some drawbacks, however: 1) it does not capture physician office costs, 2) the lack of hospital cost accounting systems necessitate the use of rough cost to charge ratios, and 3) it would not support CMS obtaining information on digital solutions that it may choose to pay for directly. However, either of these approaches would be preferable to the use by CMS of a method akin to average wholesale, acquisition, or sales price that does little to control prices. Regardless of the method, CMS needs to seek ways in which to obtain data to understand the price of the solution itself.

Practice Expenses

Practice expenses (PE) are a critical component of Medicare payment policy as they capture the direct and indirect costs associated with furnishing care, including software. For SaaS and AI-based tools in particular, PE inputs are a particularly relevant element of valuation in Medicare. A solution may reduce or eliminate distinct physician work while at the same time increasing acquisition costs and/or PE. Direct PE includes medical supplies and services, medical equipment including initial costs and peripheral costs such as software and maintenance, nonphysician clinical labor wages/benefits. Indirect PE includes non-clinical labor wages/benefits, lease/rent, office services like cleaning, nonmedical office supplies and equipment, IT supplies and services, EHR systems. CMS has historically treated SaaS as indirect PE where associated costs are not allocable to an individual patient or individual service.

As CMS has acknowledged, costs associated with software—including licensing, analysis fees, integration into existing technology, and ongoing monitoring and updates—are not well accounted for in these existing PE methodologies. Unlike a piece of equipment, an analysis fee for software does not require physical space in an office and may not require administrative staff hours to maintain it. SaaS tools also include fixed and variable components that can be more difficult to price in a way that reflects actual resource cost.

In certain cases, SaaS costs are appropriately captured in CMS's indirect PE methodology rather than incorporated as a separate direct PE input. However, survey data that informs indirect PE costs relies on data associated with software, licensing, and analysis fees with minimal costs. New SaaS tools could impact many facets of PE including staffing levels or time, power required, space needed, related supplies etc. As technology evolves, costs may no longer be minimal and existing data sets used to inform indirect PE RVUs may no longer be appropriate for use by CMS in capturing the associated costs.

A similar challenge exists in the hospital outpatient setting. Hospitals apportion indirect costs through the cost report, often distributing overhead based on square footage of a unit or other proxies that do not align with how SaaS solutions generate costs. SaaS tools generally do not drive incremental physical space, utilities, or custodial expenses in the way traditional inputs do. Instead, they may augment existing or substitute for equipment, staffing, or other direct practice resources. As a result, the current hospital cost allocation methods used by CMS risk undervaluing the resource costs of SaaS tools and misaligning incentives for adoption.

In considering valuation and how to reflect SaaS costs, CMS should ensure SaaS-related inputs are accounted for in a consistent, evidence-based manner that supports appropriate access and sustainable adoption, while laying the groundwork for longer-term reforms to unify CMS payment methodologies. To this end, we echo our comments suggesting that CMS align its approach to paying for SaaS across the PFS and OPFS.

Work

The American Medical Association (AMA) Current Procedural Terminology® (CPT) Appendix S¹⁰ sets forth a taxonomy for classifying AI tools in the context of clinical work. This framework could be used as a starting point by CMS for contemplating how clinician work valuation should be addressed in the context of Medicare SaaS payment. While the AMA's distinctions are specific to CPT coding, they can inform CMS payment policy by aligning reimbursement and oversight requirements with the level of clinical independence and associated risk. Appendix S categorizes AI-enabled services as:

- Assistive: Detects data without analysis or conclusions, like computer-aided detection imaging. Requires physician or other qualified health professional (QHP) interpretation and report.
- Augmentative: Analyzes or quantifies data in a clinically meaningful way, like continuous glucose monitoring. Requires physician or other qualified health professional (QHP) interpretation and report.
- Autonomous: Interprets data and independently generates conclusions, like diagnostics assessment for diabetic retinopathy.
 - Level 1: The autonomous AI draws conclusions and offers diagnosis and/or management options, which are contestable and require physician or other QHP action to implement.

¹⁰ <https://www.ama-assn.org/practice-management/cpt/cpt-appendix-s-ai-taxonomy-medical-services-procedures>.

- o Level 2: The autonomous AI draws conclusions and initiates diagnosis and/or management options with alert/opportunity for override, which may require physician or other QHP action to implement.
- o Level 3: The autonomous AI draws conclusions and initiates management, which requires physician or other QHP initiative to contest.

In assessing the value of work, particularly compared to existing services, this type of framework could help CMS assess efficiency gains. Something that is autonomous supplants more of the physician's work than something that is augmentative, and augmentative more than something assistive. Even with more autonomous systems, however, physician oversight, such as reviewing technology outputs, communicating results to patients, and documenting findings, should be considered in valuation. Of course, acquisition costs and changes to practice or malpractice expense could offset these gains, thus it is essential for CMS to consider all four factors in tandem.

Malpractice

CMS should also consider whether and how malpractice costs should be factored into the valuation of SaaS. Under the current physician relative value system, malpractice RVUs reflect the cost of professional liability insurance associated with furnishing a service. However, it is unclear how liability will be assigned in the context of SaaS and care delivery, whether from a regulatory or judicial perspective, which will impact the costs associated with malpractice insurance and potential judgments. For example, a SaaS diagnostic product could be flawed, which could accrue a level of risk for the physician relying on the output, the facility where the technology is deployed, and the manufacturer of the product. In addition, as SaaS tools evolve, the absence of an established framework for evaluation could make it increasingly challenging for providers and physicians to assess a technology before use or know what led to an inaccurate result. This risk is especially significant for technologies that use techniques like deep neural networks where the model's inner workings are not transparent. Moreover, as SaaS becomes more autonomous and the role of professionals decreases, there could be a shift in liability to manufacturers. Insurance, excess insurance, and reinsurance carriers will have to determine premiums that account for this lack of clarity, which may lead to higher malpractice premiums until more data and experience are available. **CMS should monitor for changes in malpractice expenses and consider whether adjustments need to be made for specialties that rely more heavily on technology.**

This challenge is particularly acute for SaaS products used outside of traditional clinical settings. Examples include mobile health applications that manage medication adherence, support cognitive behavioral therapy, or prompt patient reporting of symptoms; at-home devices such as connected blood glucose monitors or bathmats that sense the presence of ulcers; and wearable technologies like smartwatches with atrial fibrillation detection or continuous activity tracking. Reduced clinical oversight heightens the importance of accountability frameworks, oversight mechanisms, and safeguards to ensure patient safety and appropriate use. In some cases, providers may not be best suited to conduct continuing support of patient product use outside the clinical office for products with complex functionality, frequent software updates, or integration requirements. **For products used outside of traditional health care locations, CMS should**

give special consideration to direct payment to manufacturers so that there are conditions for coverage and enforcement mechanisms. CMS should have the manufacturer assume responsibility for providing technical support and patient assistance. For example, CMS should have the manufacturer offer patient-facing technical support to troubleshoot device issues, resolve connectivity problems, and provide education on correct use. CMS should also have the manufacturer provide training resources to both patients and clinicians so that the product's capabilities and limitations are clearly understood.

Data Sources

Across these four factors— Digital Solution Acquisition, PE, Work, and Malpractice— CMS should look to a variety of data sources, including publicly available information, manufacturer-submitted cost data, and other market-based inputs, to more accurately assess the costs associated with SaaS tools and determine valuation. These sources can complement Medicare claims data and provide a more holistic picture of true technology costs. Current claims data, whether public or private, do not provide sufficiently granular information. Private payers struggle with obtaining the data needed for coverage and payment analyses, as well as assessments of the quality and privacy and security of technologies. Manufacturer data agreements via pilot programs, value-based contract arrangements, and grant sponsorships for strategic emerging technologies are possibilities for private payers to obtain some of this information, but more global work by CMS to drive transparency would have a much greater impact. **Private payers recommend that CMS devise a system to create more transparency around manufacturers' costs and provider acquisition costs.**

FFS Alternatives

While we have mostly provided recommendations on how CMS should pay for SaaS within the context of FFS, the CMS Innovation Center may be a more appropriate fit for testing Medicare SaaS reimbursement given it serves as a laboratory for innovation, has the ability to waive certain generally applicable regulations to permit testing of new concepts, stipulates rigorous evaluation, and pilots time-limited models that can be halted if cost or quality concerns arise. In addition to using the Innovation Center to test the actual reimbursement of SaaS, we also encourage CMS to consider better integration of technology into APMs. Overutilization is less of a concern in models that incorporate total cost of care or shared accountability frameworks, making value-based models an attractive laboratory for experimenting with different approaches.

CMS's Innovation Center should develop risk-based contracts, for use in voluntary partnership with health plans, as a pathway for incorporating SaaS into care delivery and reimbursing SaaS solutions. As CMS has recognized in its characterization of SaaS products as indirect PE inputs, SaaS products can deliver ongoing value by improving efficiency, care quality, or patient outcomes across populations rather than being tied to a discrete clinical encounter. A risk-based contracting approach from the CMS Innovation Center would allow payers and providers to align payment with measurable performance metrics more effectively than in fee-for-service, with the goal of driving reductions in avoidable utilization, improvements in adherence, or demonstrated operational savings, while safeguarding against potential inappropriate utilization

or excessive cost growth like in per-use arrangements. A risk-based model from the Innovation Center would better reflect the longitudinal and outcomes-driven nature of many SaaS tools, while ensuring accountability and value for both Medicare beneficiaries and the program.

SaaS tools are being integrated into chronic care management and risk-based payment models to support earlier detection of disease progression, improve patient engagement, and enhance care coordination. These tools can:

- Aggregate and analyze patient-reported and device generated data between visits;
- Trigger alerts for care teams when certain thresholds are met, enabling earlier intervention;
- Support medication management and adherence monitoring;
- Provide patients with tailored education and engagement resources. Identify high risk patients, stratify populations, and support proactive interventions; and
- Expand access and empower patient self-management for lower-complexity needs, allowing clinical staff to focus more time on higher-acuity care.

One early example is Project Sonar, developed in partnership with a gastroenterology practice and a health plan.¹¹ This specialty model home used cloud-based technology to collect patient-reported outcomes from individuals with Crohn's disease between office visits. Nurses and physicians could then identify increasing or worsening symptoms early, intervene, and prevent hospitalization. The health plan provided prospective, per-patient-per-month enhanced payments to support chronic care management. Project Sonar demonstrates that SaaS-enabled remote symptom monitoring can improve outcomes for patients with chronic disease and reduce overall spending, demonstrating how software-based tools can advance outcomes under alternative payment arrangements.

CMS should leverage its Innovation Center to pilot new models of payment and care that leverage or test use of SaaS, including payment mechanisms for appropriate products. The Innovation Center could test incentives for incorporation of SaaS to support care delivery that are linked to quality measures evaluating performance.

Quality

Whether payments are made within FFS or through APMs, CMS should consider how to evaluate the quality of care associated with the use of SaaS. As noted above, SaaS tools could significantly disrupt how healthcare is delivered. Quality measures ensure that the software performs as intended in the real-world and that it supports accurate, safe, and evidence-based care rather than introducing errors or creating gaps in care. CMS should consider developing measures that assess domains such as patient safety, clinical outcomes, and patient experience. Performance measures could assess if SaaS contributes to improved outcomes on current quality measures. For example, as we establish nationwide interoperability stratify measures to look at services that rely on SaaS and those that do not. Additionally, measures could directly assess SaaS performance, including assessing patient experience with patient-facing tools. Information

¹¹ <https://aspe.hhs.gov/sites/default/files/private/pdf/253406/ProjectSonarSonarMD.pdf>.

on how use of a SaaS tool improves outcomes will promote adoption of these services as well as help monitor for safety risks.

CMS should also work with other agencies such as ASTP/ONC and the Cybersecurity and Infrastructure Security Agency (CISA) to assess other critical aspects of SaaS performance. While SaaS has great potential to improve care, it also introduces new risks to patient safety and privacy. For example, SaaS should be tested and potentially certified by ASTP to promote interoperability. Confirming information can be shared between outside software programs and a person's electronic medical record is essential to promoting care coordination, avoiding errors, and allowing a patient to have control over their complete medical record. ASTP could also update the SAFER Guides to include SaaS safety. Coordination with the FDA and other agencies will also be critical to address safety, liability, and vendor accountability as tools are continually updated.